Women's Healthcare Associates LLC

OB Manual

Patient Services

Women's Healthcare Associates

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OB Terms and Definitions

Remember to use Ctrl + F to quickly find a key term or workflow

BSUS – Shorthand for Bedside Ultrasound

TDAP - It's a booster shot that protects against tetanus, diphtheria, and whooping cough

GBS test - It's a quick swab test to see if the patient carries Group B strep. If the patient does, WHA will treat it during labor to keep the baby safe

More to come in this section!

Insight into WHA Routine OB Visits

Note: Visit timing and what is done at each appointment can vary depending on provider preference, patient history, or pregnancy needs.

OB appt quick helpful scripting – click here

1. **OB Intake Telehealth** - A phone call visit with an MA/LPN
   - Help prepare the patient for their first OB provider visit
   - Reviews early pregnancy information and what to expect
   - Discusses healthy pregnancy habits such as nutrition, exercise, and general wellness
   - Gives patients a chance to ask common pregnancy questions

2. **OB Prim Consult with BSUS - Helpful PS scripting:** *"This visit is where the patient and provider review the pregnancy, discuss testing options, and go over next steps."*

   What may happen during visit:

   - Review medical history and pregnancy details using the Bedside US machine
   - Talk about genetic screening options
     - **CF Carrier Screening** – checks if the patient carries the gene for cystic fibrosis
     - **Sequential Screening (Seq 1 & 2)** – screening done in 2 parts during pregnancy to check risk for certain chromosome or neural tube conditions
     - **NIPT** – blood test that screens for certain chromosome conditions and can also give fetal sex if desired

- Discuss where the patient plans to deliver
- Place ultrasound orders if needed
- Answer patient questions
- Common genetic screening options discussed

3. **Dating and Viability Ultrasound -**
   - Confirms dating and viability of the pregnancy
   - Gives the provider updated information

4. **OB Workup - What may happen at this visit:**
   - Review ultrasound results
   - Breast exam if needed
   - Pap smear if due and/or desired
   - STI testing
   - Urine sample to check for infection

5. **Labs - Common labs may include:**
   - Blood type
   - Iron levels
   - HIV and other routine prenatal labs

Helpful PS scripting if patient is being seen for these visits between 12 – 13.6 wks: "*These visits help gather the routine early pregnancy information and testing needed to support prenatal care.*"

6. **16-week OB Check:**
    - Belly measurement may begin this week (to track how baby is growing)
    - Addressing patient questions
    - Discussing future visits

7. **Anatomy Ultrasound + 20 week OB Check:**
    - Detailed anatomy ultrasound to check baby's growth and development
    - May include finding out fetal sex if Ultrasound tech is able to determine and if the patient wants to know
    - Provider visit to review ultrasound results and answer questions

8. **OB Check at 24 weeks:**

What may happen at this visit:

- Glucola drink may be given for an upcoming lab test
- Additional labs may be ordered
- Urine sample may be collected
- Baby's heartbeat may be checked
- Belly measurement may begin around this point in pregnancy (this is a routine way providers track how baby is growing)

9. **OB Check at 28 weeks + lab visit - Helpful PS wording:** *"This is one of the bigger routine pregnancy visits because it often includes important lab work and vaccines."*

What may happen at this visit:

- Glucose testing/lab work is commonly done around this time
- Routine pregnancy follow-up
- Depression screening may be completed
- TDAP vaccine may be offered/recommended
- Urine sample may be collected
- Baby's heartbeat may be checked if indicated
- Belly measurement continued

10. **OB Checks at: 30 weeks, 32 weeks, and 34 weeks - Helpful PS wording:** *"These are regular follow-up pregnancy visits to monitor both patient and baby as the pregnancy progresses."*

What may happen at these visits:

- Vaccine may be offered between 32 – 36 weeks to prevent respiratory virus
- Review symptoms, concerns, or questions
- Check baby's heartbeat
- Urine sample may be collected
- Measure belly growth
- Review baby movement, labor precautions, and next steps as pregnancy progresses

11. **OB Check at 36 weeks - Helpful PS wording:** *"This visit includes some of the routine preparation for delivery and postpartum planning."*

What may happen at this visit:

- Routine screening completed around this time as part of delivery preparation (GBS swab)
- Discuss breast pump needs
- Discuss postpartum birth control options
- Cervical check may be offered
- Routine heartbeat check, urine sample, and belly measurement

12. **OB Check at: 37 weeks, 38 weeks, and 39 weeks - Helpful PS wording:** *"These are regular follow-up pregnancy visits to monitor both patient and baby as the pregnancy progresses."*

What may happen at these visits:

- Check in on how patient is feeling
- Review symptoms, concerns, or questions
- Check baby's heartbeat
- Urine sample may be collected
- Measure belly growth
- Review baby movement, labor precautions, and next steps as pregnancy progresses

13. OB Check at 40 weeks - Helpful PS wording: *"This visit focuses on the plan moving forward"*

What may happen at this visit:

- Discuss next steps if the patient goes past their due date
- Cervical check may be offered
- Routine heartbeat check, urine sample, and belly measurement

Quick Guide - Why is this visit important?

- **OB Intake Telehealth:** "This helps get you started and prepares you for your first clinician visit"
- **OB Prime Consult:** "This is where you meet with the provider and review the pregnancy plan"
- **Ultrasound/Workup/Labs:** "This visit helps complete the routine testing and information needed in pregnancy"
- **24–34 Week Visits:** "These are routine follow-ups to monitor you and baby"
- **36 Week Visit:** "This visit includes important delivery prep"
- **40 Week Visit:** "This visit focuses on next steps forward"

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Ultrasound Guide

WHA Ultrasound Scheduling Guide effective 11/2025		
OB US Order Name	**Singleton Time**	**Twins Time**
Dating/Viability - *(≤ 13.6 wks) - Do NOT schedule from 7-8am*	50	50
LIMITED or DETAILED First tri. anatomy w/ NT - (12-13.6 wks) - *Do NOT schedule from 7-8am*	50	60 (*if known Twins)
Sequential screening - (12 - 13.6wks) - *Do NOT schedule from 7-8am*	50	60
Dating/viability - (≥ 14.0 weeks) - *Do NOT schedule from 7-8am*	60	120 (*if known Twins)
TVCL (Transvaginal Cervical Length) only (16-24 wks)	50	50
Anatomy (≥ 14 wks) Routine *or* Detailed	60	120
-Including TVCL (≥16 wks- 24 wks)	70	130
F/up anatomy - limited (suboptimal views)	50	60
- Including TVCL (≥16 wks - 24 wks)	60	70
F/up anatomy and/or growth - *no prior* WHA *Anatomy* study	60	120
F/up growth / Fup anatomy + growth	50	60
- Including BPP	50	60
F/up placental location - Including transvaginal (≥32 wks) (Low lying placenta, placenta previa, vasa previa, placenta accreta)	60	70
Mono/di twin early anatomy (16 wks)/TTTS surveillance		120
Mono/di twin f/u anatomy/growth (20 wks)/TTTS surveillance		60
Mono/di twin TTTS surveillance - routine		50
Mono/di twin surveillance - routine w/ growth and/or BPP		60
BPP only	50	60
AFI only	50	50
Fetal position	50	50

GYN US Order Name	Time	Special Instructions
Pelvic US	60	Schedule follow up Provider appt 3+ days out
US guided GYN Procedure (IUD insertion/removal)	60	MD to join no sooner than 15-30 mins into exam, for 30 min
US guided GYN Procedure (endometrial biopsy)	60	
SIS w/complete pelvic US done in last 6 months	70	

Helpful Contacts:	What they can help with:
US Scheduling Line	Help Schedule
NWP US Scheduling Coordinator CQ Line	Approval for Spots

Call NWP Scheduling before scheduling on a NWP US room

Call NWP PS Team if you have an Ultrasound Order that is not listed - Example: Fetal Echocardiogram

Important Note :

If a patient is having their first US with WHA and they are ≥ 14 weeks they need a 60 min US unless more time is notated in guide

Color Key :

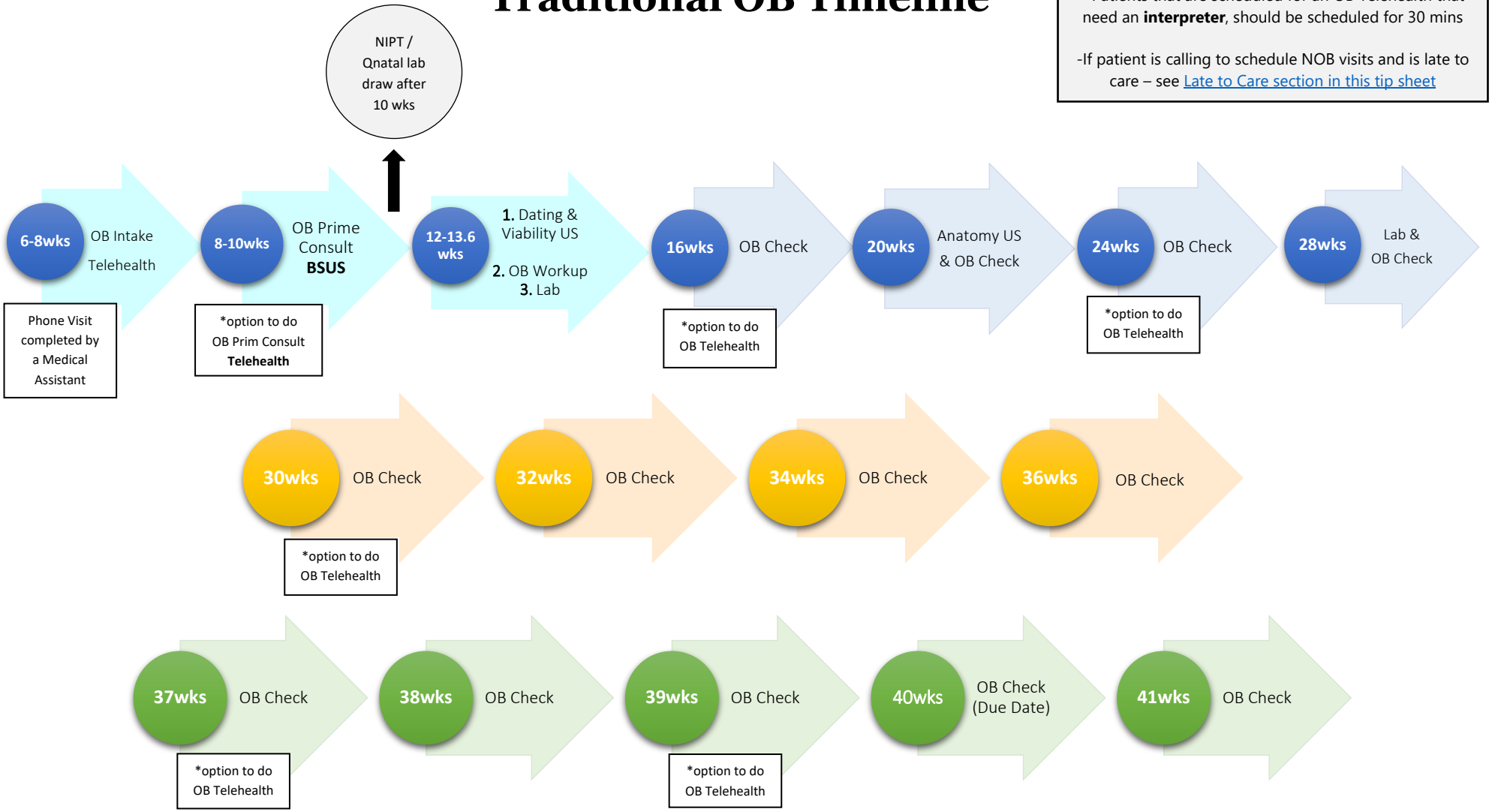
OB Complications (Cramping/Bleeding) and exams indicated by this color require a same day follow up appt with a Provider. These exams can also be scheduled together as one exam

Traditional OB Timeline

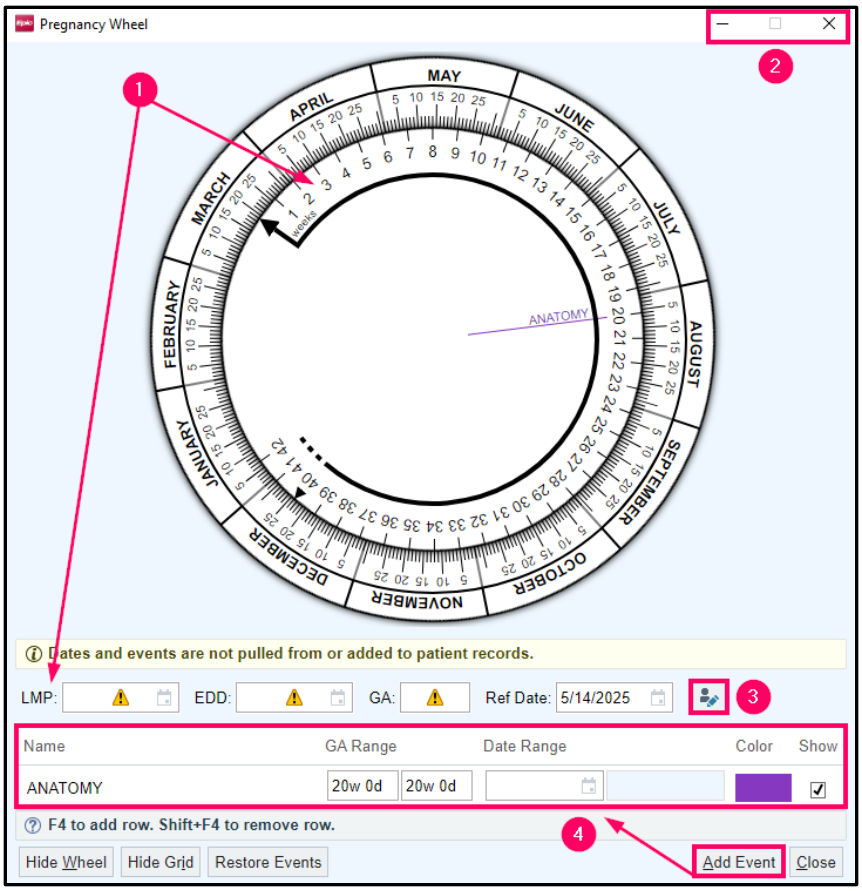
Note:
- Need to leave a 30 min gap after **each OB US** before the provider visit

- Patients that are scheduled for an OB Telehealth that need an **interpreter**, should be scheduled for 30 mins

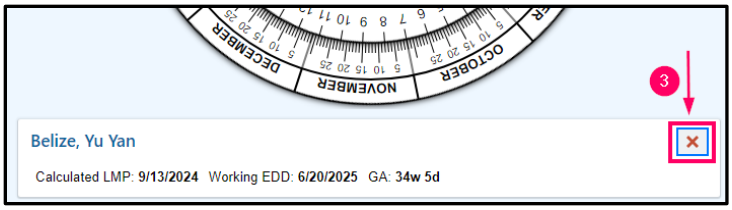
-If patient is calling to schedule NOB visits and is late to care – see Late to Care section in this tip sheet

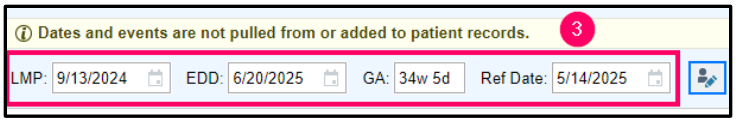


Setting up the Pregnancy Wheel









1. Click and spin the wheel weeks to update the LMP/EDD/GA dates or add this info manually under the yields (*Example: Adding the LMP will calculate the EDD & GA*)

2. The wheel opens as a **separate Epic window**. It can be minimized during scheduling or any other work in the background. Select the window from your desktop task bar to pull it back up when needed. Click the X to close

3. If the patient has current pregnancy info under their chart, it can be pulled in.
   - Click the **select patient** button to look up and pull in the patient. Once the info is pulled in, make sure to click the **red X** to utilize all the features.
   - If using the Pregnancy Wheel from the **Chart view**, this info will auto pull in
   - Use the **Ref Date** after scheduling to calculate GA at time of the visit when applicable

4. **Events** can be added along with the GA Range to help with scheduling
   - Choose a **color** and click the **Show box** for the event appear on the pregnancy wheel

Medicaid / OHP Fertility Scheduling Workflow

- Not a covered service if OHP is primary or the patient's only insurance
- If/when a patient requests to be Self-Pay for this non-covered service we can honor a self-pay transaction. However, WHA is **not** allowed to suggest or offer the option of being self-pay – ask the FC team for more information
- WHA does NOT do TUBAL REVERSALS!!! Do **NOT** schedule consults for this

When scheduling, utilize this scripting:

"Fertility/Pregnancy planning may not be a covered service with your insurance plan. We will have our financial department review your records to determine that and they will reach out to you if we are unable to see you due to the reason for your visit."

Note: This turnaround time is about 1 week. The FC team will call the patient if the visit is not covered and will send a Myhealth message if it is covered.

It is important to add detailed appt notes to the patient's visit to help the FC team determine covered benefits:

- **Planning for Pregnancy** = schedule visit and add appt notes to indicate Planning for Pregnancy. Then send Staff Message to the appropriate FC Pool for benefits check using dot phrase below

Dot phrase to use: .WHAPSOHPFERTILITYSCHED

"OHP patient scheduled for fertility visit. Please review records for visit reason and reach out to patient to let them know if services are covered or not"

Scheduling New OB Visits

For the Scheduling Bedside US workflow tip sheet – click here

- OB Intake Telehealth visits will be done by a designated medical assistant, LPN, or RN assigned to the Tualatin Prenatal Intake Resources (Schedule A, B, C, or D) using the **OB INTAKE TELEHEALTH VISIT** (defaults to 30 minutes)

- Prenatal Intake Video via MyHealth - *see full workflow for scheduling and scripting*

- **OB INTAKE EXTENED TELEHEALTH** block type will be used for patients needing interpreters or have technology/other barriers to accessing Myhealth (defaults to 75 minutes) – **use Manual Search!**

- **All OB patients** will need to have an OB Intake Telehealth visit initially (this will be on the Tualatin Template)

- If a patient is opting to schedule their routine **12 week Ultrasound at an outside WHA facility**, the PS member needs to add "Obtaining 12 week US externally" in the Prenatal Intake appointment notes. This is to indicate to the MA facilitating the Prenatal Intake visit to have a conversation with the patient that they can't get the NT SEQ genetic testing unless they reschedule to have their 12 week Ultrasound with WHA

- **ER Follow up NOBs –**

1. Update patient registration or register completely if patient is new - Registration update must be completed before moving on to the next step
2. Start and complete OB Intake telephone encounter
3. The scheduler should **not** schedule the patient for an OB Intake Telehealth or any other NOB visits
4. Route completed encounter to clinic's OB coordinator pool
5. Create a separate Telephone Encounter. This should include the following:

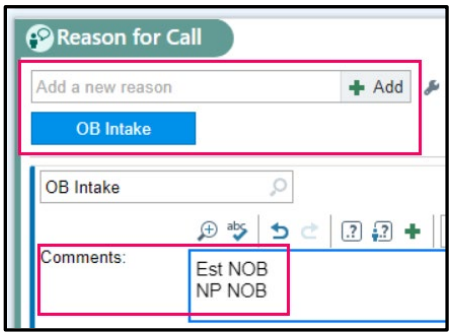
- Reason for Conversation = ER Follow Up
- Utilize Smart Phrase = **.WHAEDFOLLOWUP**
- Route to the Advice Pool at the site where the patient desires to be seen**.** Let the patient know that a nurse will be calling them back to discuss

New OB Quick Summary Walkthrough

If you have any questions about this workflow, contact your OB Coordinator

- Open the patient's **Appt Desk** (if brand new WHA patient, fully Register them)
- Start a **Telephone Encounter**
    - Fill out each section: **Contacts, Reason for Conversation, Documentation and Routing**

Note for Reason for Conversation: Must be OB Intake for the Reason and NP NOB or Est NOB for the Comment



Note for Documentation: Use the NOB Intake SmartPhrase in Documentation

- When asking NOB questions, if patients have any medical concerns, then either start a second Telephone Encounter explaining the specific concerns and routing to the Advice Nurse Pool **or** call an Advice Nurse for more urgent needs

- Determine the patient's estimated Gestational Age from their LMP using the **Pregnancy Wheel**

- **Route and Send the Telephone Encounter** to the appropriate clinic OB Coordinator Pool

- Go back to the **Appt Desk** tab

- Schedule the following appointments using the patient's LMP/GA *(use Telehealth Guide for Telehealth Appt Notes):*

    - 6-8 week **OB INTAKE TELEHEALTH VISIT** Use the TN Department it will pull in the TN Initial Prenatal Appt resources A,B,C, & D
      (*This phone visit is done by a Medical Assistant*)

    - 8-10 week **OB PRIME CONSULT BSUS Visit OR OB PRIM CONSULT TELEHEALTH** (*this visit is the first with a clinician*)

    - 12 -13.6 weeks **Dating & Viability US / OB Workup / Lab Visits**

- Sign up patient for **Myhealth,** if needed

- Add the patient's **Care Team** member(s). The patient's Care Team provider needs to be the provider the patient and the scheduler identify as the patient's preferred provider for pregnancy (do **not** default to the provider the OB Workup is scheduled with)

- Add Appt Notes if you haven't done so already

- If you schedule a patient for an OB Intake Telehealth within 24 hours, send a patient Myhealth message using the dot phrase .WHAPIVIDEO

New OB Full Walkthrough

Step 1: OB Intake Telephone Encounter

- Scheduler creates OB Intake Telephone Encounter using **.WHANOBINTAKE** and routes to site's OB Coordinator Pool
    - Use .WHANOBINTAKE dot phrase:

Patient called wanting to establish New OB Care. Discussed the following questions with the patient:
Last Menstrual period or EDD Information: {WHA LMP DATE:44611}
When was your first positive pregnancy test? ***
Have you been seen by any other clinic for this pregnancy? {Yes No - OB transfer:41216}
Have you had any ultrasounds with this pregnancy? {WHA New OB Intake Ultrasounds:42168}
What number child is this for you? ***
What past delivery types have you had? {WHA Delivery Types:58194}
What is your preferred delivery location? {Del loc clinic:44049}
What is your ideal location of care? ***
Do the preferred delivery location and the ideal location of care match? {Hospital and Clinic yes no:44094}
Do you have a specific provider you would like to start care with? ***

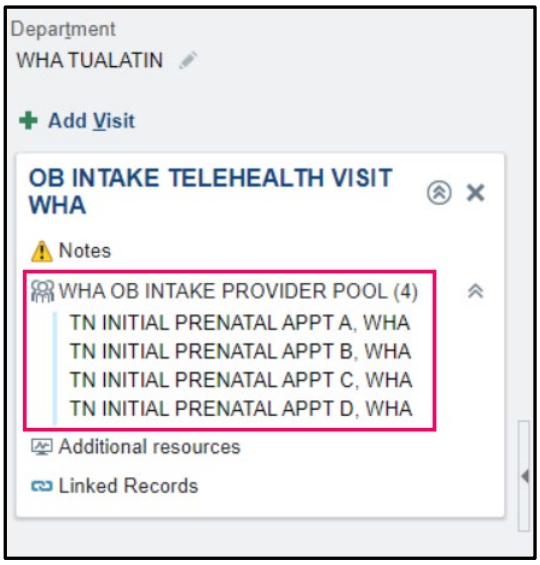
Please read this to the patient before ending your call: If you have any questions or concerns that come up before your appointment, you can call us, we have advice nurses available from 8:30am-8pm Monday through Friday, and Saturdays 8a-5pm.

- If patient indicates they are unsure of their LMP date (cannot give you even an approximate) then you can connect them with our Advice team – do this via **Warm Transfer** on the phone

- If patient indicates that they are unsure of next steps, offer scheduling routine prenatal care or options counseling.
    - Scripting: *Would you like to schedule routine prenatal care?* Pause…*or a visit to discuss your pregnancy options?*
    - If options counseling desired see options counseling workflow

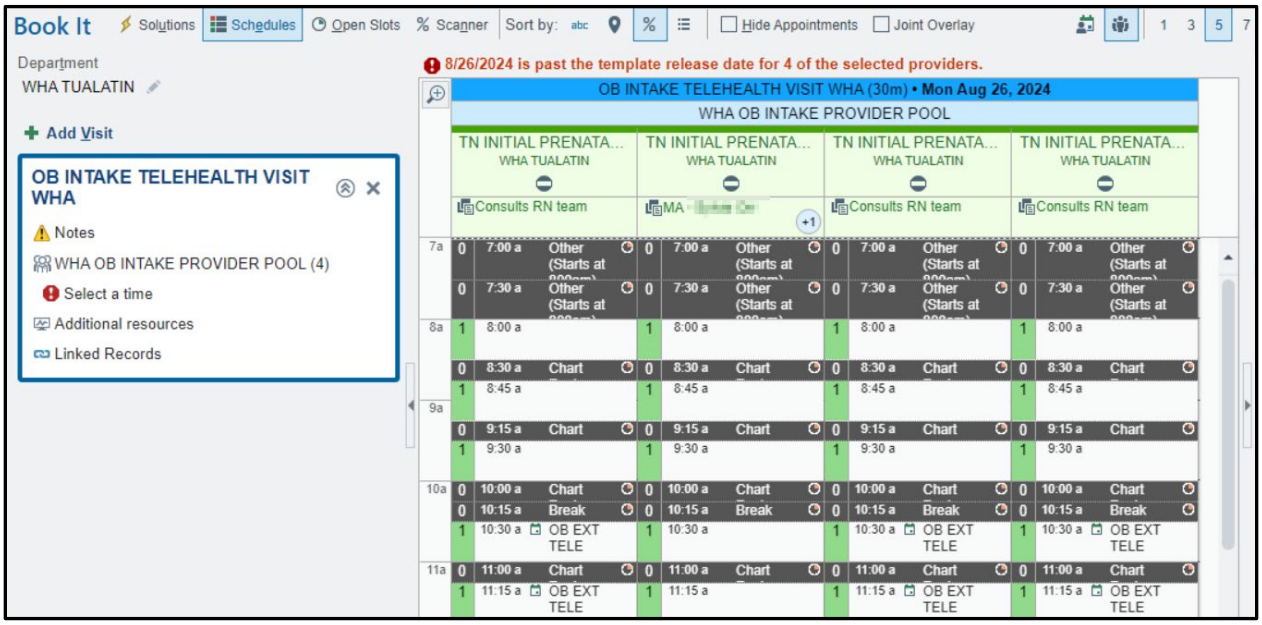
- If a patient is opting to schedule their routine **12 week Ultrasound at an outside WHA facility**, the PS member needs to add "Obtaining 12 week US externally" in the Prenatal Intake appointment notes. This is to indicate to the MA facilitating the Prenatal Intake visit to have a conversation with the patient that they can't get the NT SEQ genetic testing unless they reschedule to have their 12 week Ultrasound with WHA.

Step 2: Schedule OB Intake Telehealth



- Use this scripting: "*To start your care, we will schedule you for an intake with a clinical team member. We will be sending you a link via your MyHealth account – be sure to activate and check your account prior to your appointment. This video is 30 minutes, please make sure to watch it before your first scheduled appointment.*"

- **OB INTAKE TELEHEALTH** visit type is defaulted to 30 minutes. It will automatically pull in only the Tualatin Prenatal Intake Resources



For patients **who need an interpreter,** manually search for the block type OB INTAKE EXTENDED TELEHEALTH on the Tualatin Template. There will be one OB INTAKE EXTENDED TELEHEALTH appt per day, per Resource (usually around 10:30am or 3:00pm) & it will thaw off 48 hours prior if not filled. Use BOTH OB INTAKE EXT TELEHEALTH spots – creating a 75 minute long appt for these patients.

For patients that have **technology limitations and/or are not able to access MyHealth**, schedule them as a normal OB Intake Telehealth (30 minutes).

Schedule visits top down, offering the first available appointment in the morning or after lunch, to ensure patients are back to back with no open space between them

Step 3 – OB Prime Consult BSUS Visit OR OB Prim Consult Telehealth

For the Scheduling Bedside US workflow chapter – click here

Ideally this visit would be scheduled with the same provider that will be scheduled to do their OB Workup. The 2nd option is a provider within the preferred provider's call group.

If patient is declining this visit, utilize the following scripting:

- *"In order to proceed with your 12 week ultrasound and in person provider visit, we must ensure that you complete this appointment. This appointment is necessary for your provider to evaluate your pregnancy and ensure we have the correct ultrasound order in place for the 12 week ultrasound and in person provider visit."*

Step 4 – Schedule OB Workup Visits

The Dating and Viability ultrasound/ OB Workup / lab should be scheduled between

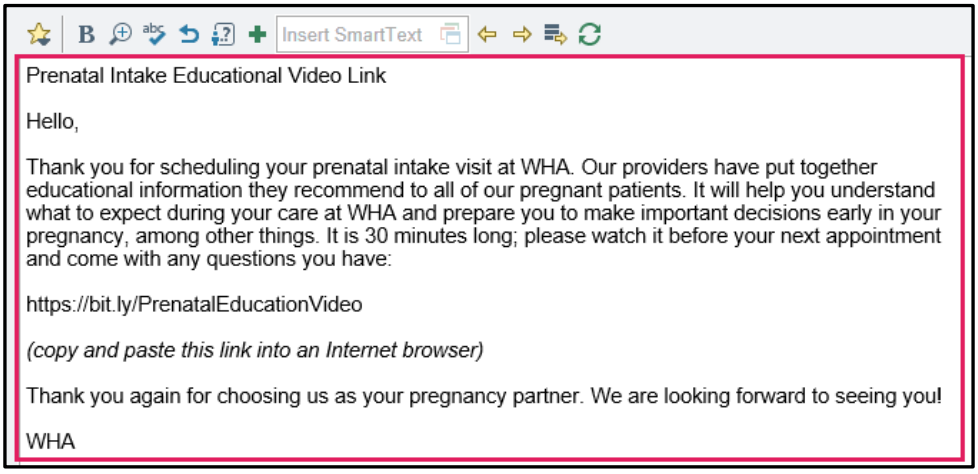
12.0 - 13.6 weeks

- **Remind patient to activate/check MyHealth –** messages are sent the following business day for all appointments that are not next business day
- If you schedule a patient for their OB Intake Telehealth (on the Prenatal Tualatin Resource) within 24 business hours, send the patient a MyHealth Patient Message using the specific dot phrase

Steps on how to send Myhealth Patient Message:

- Open a new Patient Message within the In Basket screen
- Use PI video smart phrase, which is: **.WHAPIVDEO**
- Check mark: "Do not allow patient reply"

- Click send (bottom right corner)



- If a patient comes to the OB Intake Telehealth appointment without watching the video the MA/LPN will still complete the appointment. They will additionally encourage the patient to watch the video prior to their OB Primary Consult Telehealth appointment.

If unable to schedule patient within time frame of 12.0 wks - 13.6 wks - Click here

Step 5 – Care Teams

Enter the Care Team provider in the patient's registration:

The patient's Care Team provider needs to be the provider the patient and the scheduler identify as the patient's preferred provider for pregnancy (do **not** default to the provider the OB Workup is scheduled with)

Two WHA Care Team providers need to be entered at this time:

1. The patient's preferred provider for this pregnancy
    a. Relationship listed as **Obstetrician**
    b. Start date of date patient informed us they are pregnant

c. End date is patient's rough 3 week postpartum date

2. The patient's preferred provider for GYN care:
    a. Relationship listed as **Gynecologist**
    b. Start date is 1 day after their listed "3 week postpartum" date
    c. No end date

If there are any past WHA providers already listed as Gynecologist or Obstetrician, you will need to expire these to keep the patient's Care Team as accurate as possible

When scheduling all future OB visits, schedulers should be reviewing the Care Team with the patient to ensure it is accurate. If a patient or scheduler identifies that the Care Team entered is no longer who the patient is preferring to see for their pregnancy, the scheduler will update the listed Care Team members and send a Staff Message to the Care Team provider's Pod to update the pregnancy episode of care if indicated

FAQs:

- All patients should be strongly encouraged to activate and login to their MyHealth and shared the importance of watching the video prior to their MA/LPN OB Intake Telehealth on the Prenatal Resource on the Tualatin Template

- Each TN Prenatal Intake Resource will have one 75 minute spot per day for patients who need an interpreter or have technology limitations and cannot watch the video prior. Use the **OB INTAKE EXTENED TELEHEALTH** block in the Schedule screen

- Each morning messages are sent to the patient who called the prior business day and scheduled an OB Intake Telehealth (no matter how far out the visit is)
    - If you schedule a patient for their OB Intake Telehealth (on TN or Nurse rooming) within 24 business hours, Send the patient a MyHealth Patient Message using the dot phrase *(see step 4 for more info)*

- If a patient comes to the OB Intake Telehealth appointment without watching the video the MA/LPN will still complete the appointment. They will additionally encourage the patient to watch the video prior to their OB Primary Consult Telehealth appointment.

Q: What if a patient calls with an unknown LMP?

Answer:

- Follow this chapter through all the steps

Q: If a patient calls with a genetic testing decision?

Answer:

1. PS team to look for telephone encounter labelled **"Other - Genetic testing decision follow up"**
    a. If telephone encounter is found, they will document patient decision in this telephone encounter and route to the site OB Coordinator pool

    b. If no telephone encounter labelled "**Other - Genetic testing decision follow up"** can be found**,** PS team member to look for **OB Intake telephone encounter** and will document patient decision in this telephone encounter and route to the site OB Coordinator pool

Q: What if I can't get the patient in for their Dating & Viability ultrasound within the 12.0 wks – 13.6 wks timeframe?

Answer: It depends on the reasoning.

It is ok to schedule at WHA outside 12.0-13.6 wks, if the reason is:

- There is NO access at WHA then it is ok to schedule at 11.4 – 11.6 wks

- Patient will be out of town then it is ok to schedule at 11.4 – 11.6 wks
- If the Provider says it's an ok option

If you do schedule within the 11.4 – 11.6 week timeframe for the Dating and Viability US, send a Staff Message to the US scheduling Pool **and** your sites OB coordinator pool indicating the reason why patients are scheduled prior to 12 weeks

If there isn't any access at WHA during 11.4-11.6 wks, click here for the outside imaging order workflow

If scripting is needed:
We schedule your 12 week visits between 12.0 weeks and 13 weeks and 6 days of pregnancy as this is the ideal time to get the best and most detailed first trimester images. The images we collect in the 1st trimester are determined by our AIUM accreditation, which is an accrediting body of excellence in ultrasound with specific standards that we must meet.

Scripting if unable to schedule within the 12.0 wks – 13.6 wks time frame:

We could look at doing this visit just a couple days earlier than the recommendation, we do want you to be aware that if you opt for the Sequential screening ultrasound a scan before 12 weeks may not get the results needed.

If patient cannot schedule within recommended time frame or sooner:

We do not schedule second trimester ultrasounds during this initial phone call; we can get you scheduled for a visit with your provider past the 13.6 week time frame and they can review with you the need for a 2nd trimester ultrasound and order one if it is needed prior to the routine 20 week ultrasound.

OB Coordinator Role within NOB visits

1. Reviews Telephone Encounter and scheduled visits to ensure accurate scheduling

    - OB Coordinator to review NOB Intake telephone encounter question 4 regarding **records, past care, and ultrasound**
    - After review, the OB Coordinator will enter the information below into the Intake Encounter and then defer it out to the day after patient's scheduled OB Intake Telehealth visit.

      Add:
        - OB Intake Telehealth date
        - 8 week visit date – (8-10 week BSUS date or 8 week telehealth)
        - OB Workup date - (Formal US and OB Workup)

2. Postpone Telephone Encounter to the day after Intake to ensure the visit was completed

- If the Intake **was not** completed, call the patient 3 times prior to 8 week OB Primary Consult Telehealth **or** OB Prime w/ Bedside US to attempt to reschedule the intake
- If the Intake **was completed** ensure 8 week (OB Primary Consult Telehealth **or** OB Prime w/ Bedside US) and 12 week visits are still scheduled

3. If patient **has not completed** OB Primary Consult Telehealth or OB Prime w/ Bedside US:

- OB Coordinator will attempt to call the patient 3 times to reschedule the 8 week visit to either an OB Prime w/ Bedside US or an OB Prim Telehealth (whichever visit can be scheduled before the 12 week visit)
- The process is to call the patient once per day for 3 business days. If the patient has a Myhealth, also send them a message asking them to call to reschedule their

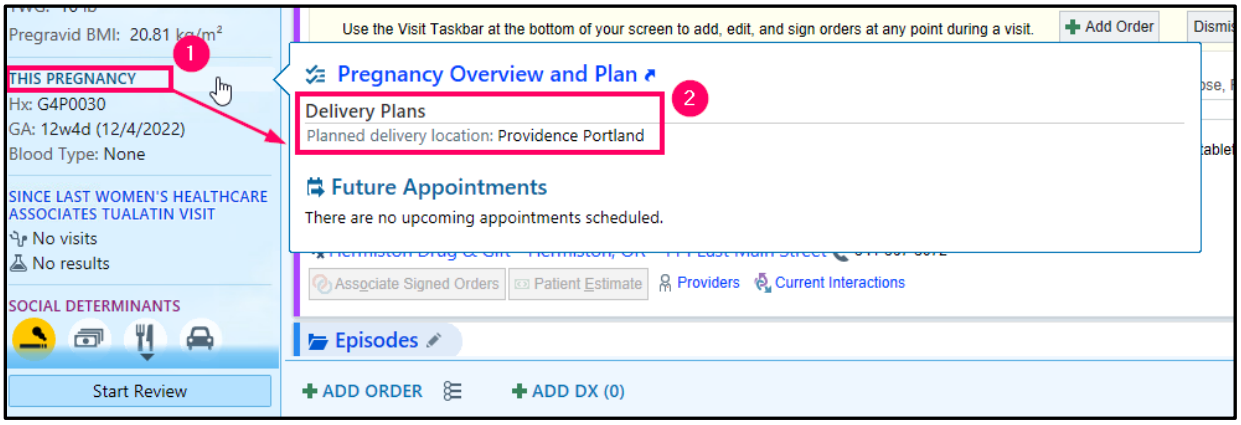
OB visits. When calling for the 1st time to reschedule missed 8 week visit if the patient's phone is disconnected or wrong and they do not have an active Myhealth account, mail them a letter via USPS mail

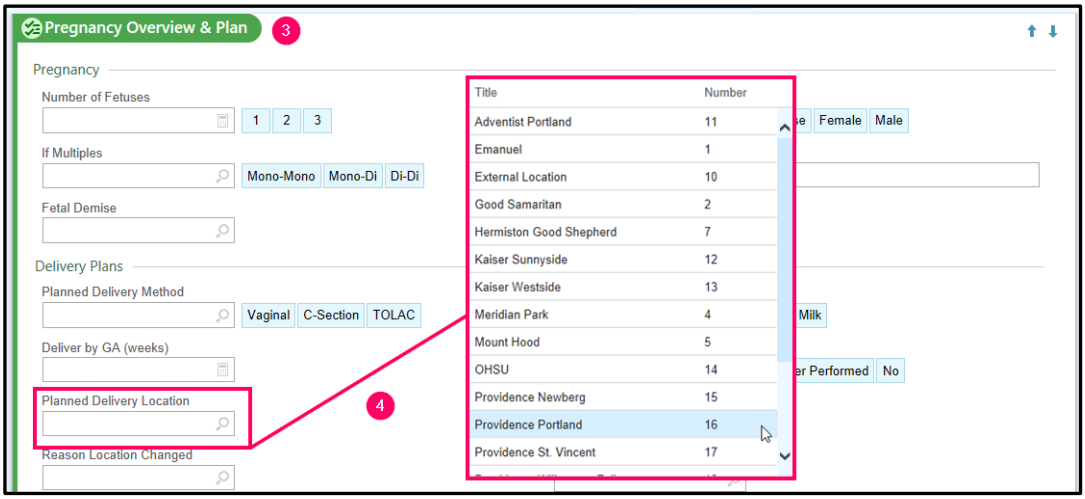
- If unable to reach patient, postpone the OB Intake Telephone Encounter to 2 days prior to 12 week ultrasound
- 2 days prior to the 12 week ultrasound, make a 4th and final attempt to call patient. If unable to reach the patient to get rescheduled, cancel all 12 week visits.
- Update the OB Intake Telephone Encounter and create a Patient Message on the Appt Desk notating why the visits were cancelled and instructions for the PS team on how to reach out to the OB Coordinator if patient calls back

If patient **has completed** OB Primary Consult Telehealth or OB Prime w/ Bedside US:

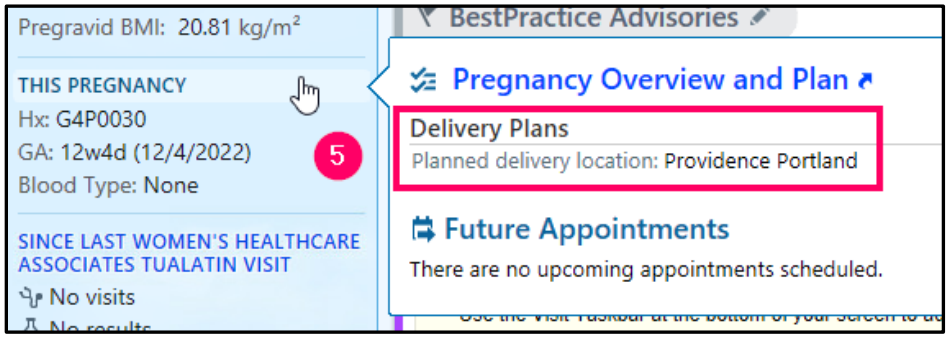
- Go to the Appt Desk to verify there is an ultrasound order and that it is linked to the 12 week Ultrasound order (link to US if not linked)
- If there is not an ultrasound order, the OB coordinator will Staff Messages the provider that did the 8 week visit to enter an US order.
- If no response or action is taken from the Provider, send another Staff Message to them
- If a 3rd request for US order is needed the OB coordinator will send the request to the provider's POD pool
- Verify the 12 week visits are all scheduled correctly (timing and order of visits)

- **Checking and finding the planned delivery location**





1. Under patient Storyboard in the Chart, hover over **This Pregnancy** to expand the Pregnancy Overview and Plan
2. If the delivery location has been entered it will appear under **Delivery Plans**
3. If no delivery location is listed but you find the information documented elsewhere in the chart, click on **This Pregnancy** to open the overview
4. Go to **Planned Delivery Location** and select the delivery location from the list



After entering the location, continue any work being done in the OB Intake telephone encounter or documentation. There is no need to sign the encounter as navigating away from the Pregnancy Overview and Plan section will automatically save the location entered

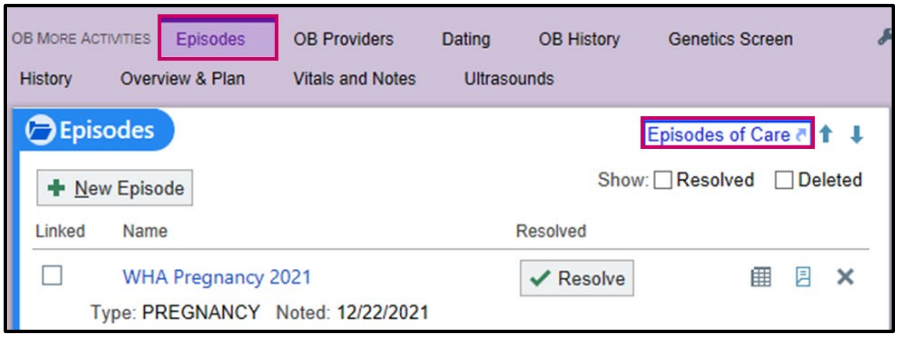
5. Hover over **This Pregnancy** again to check if the location was added correctly

Note: If you see "**No pregnancy episode available**" do these steps below:

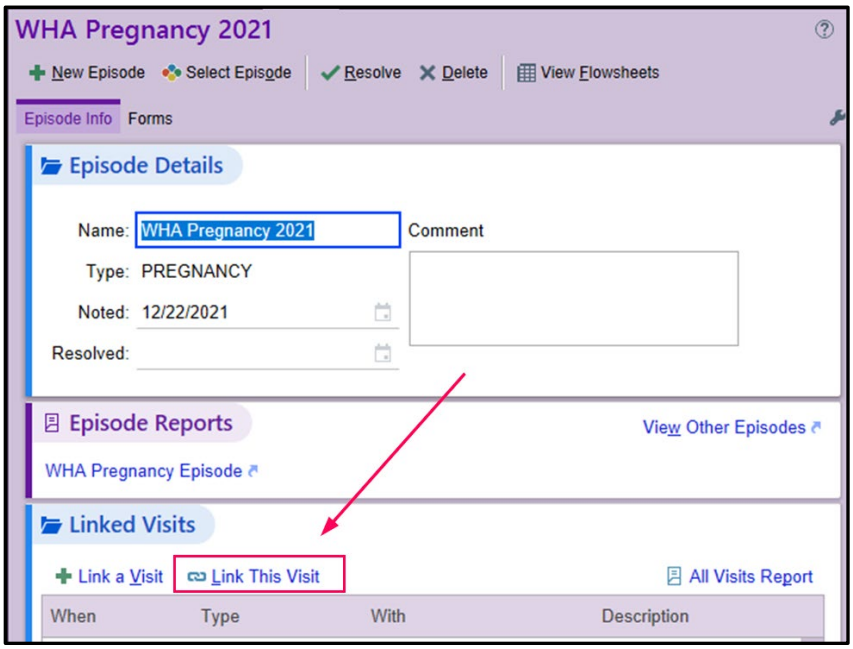
If you see "**No pregnancy episode available**." click Close. This occurs because the Telephone Encounter wasn't linked to the episode when the episode was created.



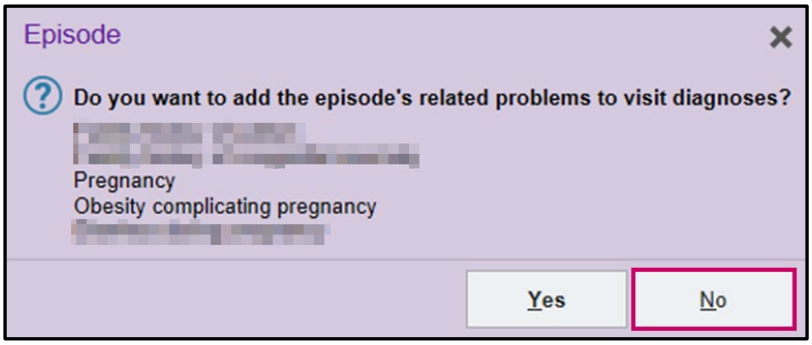
To fix that, go to the **Episodes** activity and click the hyperlink **Episodes of Care**



Click **Link This Visit**



If you get a pop up asking to add related problems to visit diagnosis, **click NO**



There will be a slight delay after clicking back into the pregnancy overview and plan, but when it loads you will be able to add the patient's delivery location. From here you can do the rest of your workflow

Note: You will have to do this for most patients because of the nature of the workflow. Once the encounter gets linked to the episode you **won't** need to link it again.

4. Postpone to after the 12 week visit to ensure patient had visit or reschedule if needed

5. Once 12 – 13.6 week visit is completed, sign the OB Intake encounter

Note: If the patient miscarries during the first trimester, OB Coordinator will send a staff message to WHA Billing & Coding Pool indicating end of pregnancy. OB Coordinator will document in the telephone encounter that a staff message was sent to the WHA Billing & Coding Pool then sign off the telephone encounter

Financial Coordinator Role within NOB visits

1. Runs an EPIC WQ indicating if patient is scheduled for NOB visits to obtain benefits
2. Verifies benefits and contacts patients with insurance coverage
3. Financial Coordinator enters deposit on patient's account for collection at the appropriate gestational age

Bedside OB Ultrasound Scheduling

For entire NOB workflow not related to Bedside Ultrasounds and/or how to schedule the OB Intake Telehealth *(including the OB Coordinator's role pertaining to Bedside OB Ultrasounds)* – See chapter Scheduling New OB Visits including initial Telehealth

NOB Visits w/ Bedside OB Ultrasound Workflow

Summary: For the clinics with Clinician's participating in OB Bedside Ultrasounds for NOB patients, the PS team will schedule their patient's second NOB visit differently

Step 1: Scheduler gets patient on the phone, starts & complete OB Intake Telephone Encounter using NOB Intake SmartPhrase. If patient knows their LMP or has an approximate date, send and route Telephone Encounter to OB Coordinator Pool

If the patient has an unknown LMP – follow the step under this section: Unknown LMP

Step 2: Next, the PS team member will schedule these visits using the patient's LMP date:

OB Intake Telehealth Visit on the TN Initial Prenatal Resource - (6-8 weeks)

The OB Prime Consult BSUS WHA visit type (this will include a Provider and Bedside US Resource) – (8-10 weeks is preferred but can scheduled up to 12 weeks)

Helpful Scripting: "*Your second visit is a provider performed Ultrasound to confirm your Dating"*

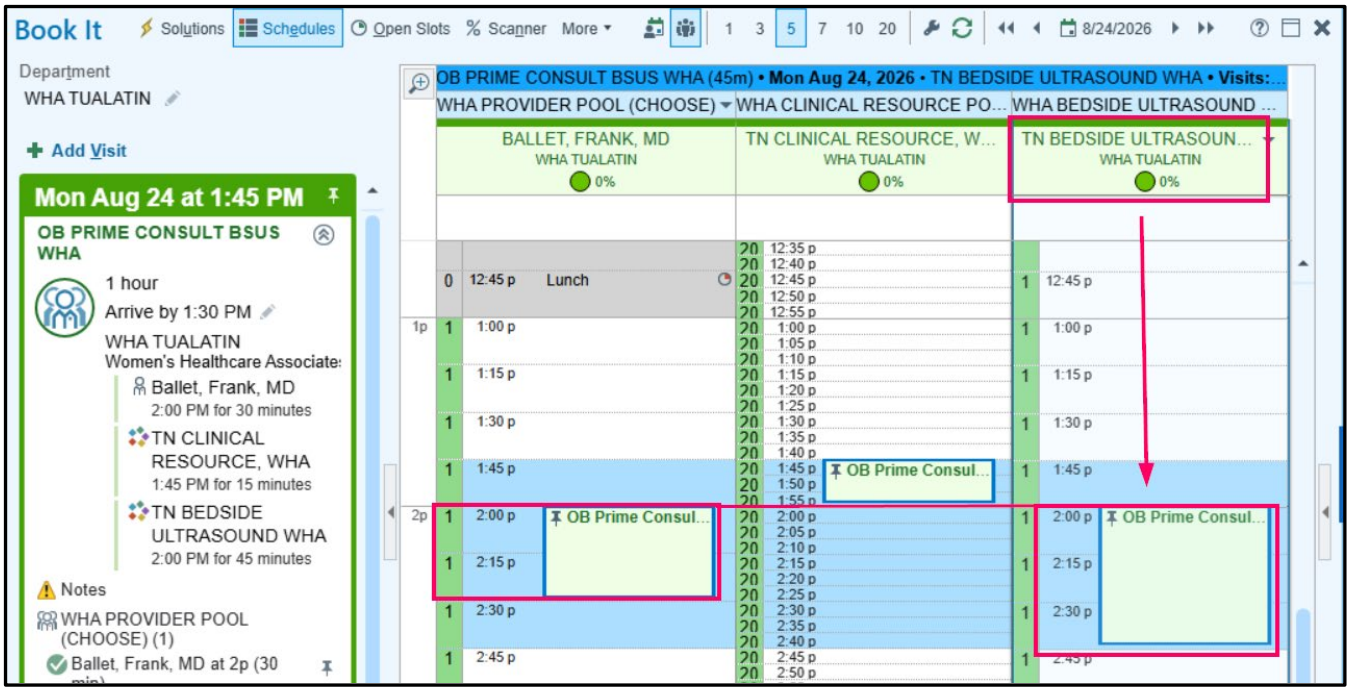
And last schedule the Dating & Viability Ultrasound /OB Workup /Lab visits (12 – 13.6 weeks)

Bedside Ultrasound FYI's

- Orders will be placed by the Clinician during the OB Prime Consult BSUS visit if needed

- Utilize the Solutions scheduling window to make scheduling the OB Prime Consult BSUS visit easier, this will automatically search for available Providers and available Bedside US Resource time

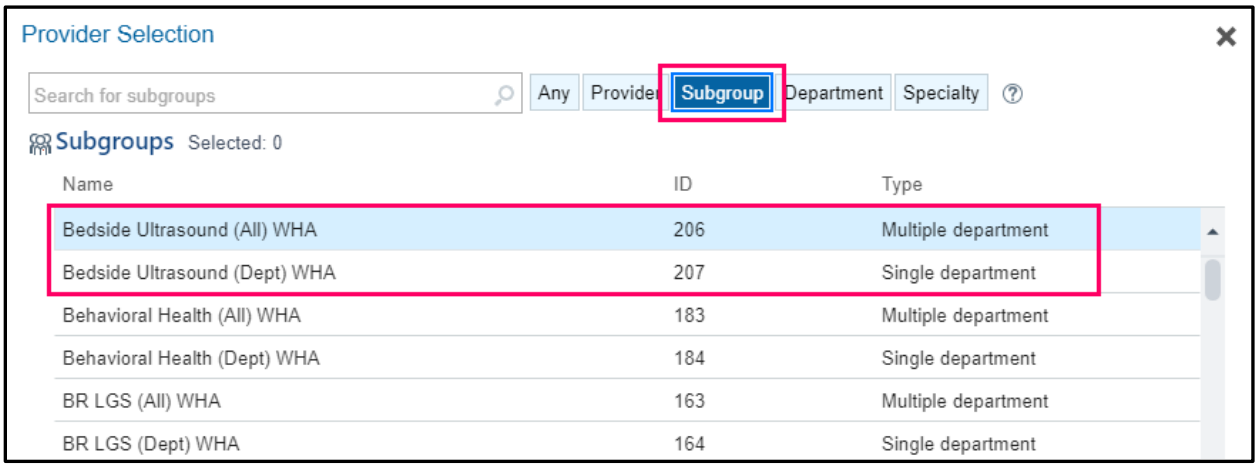
- If using the Schedules view within Book It for the BSUS visit, be sure the BSUS resource is lined up with the start time of the Provider visit – not the rooming Clinician Resource. The BSUS visit will extend past the Provider time – this is to give the staff time to clean the machine before the next usage



- The OB Prime Consult visit with Bedside Ultrasound MUST be scheduled at the patient's home location with preferred provider/provider group

- Review bladder prep Ultrasound instructions (under patient instructions within Epic) – these are the same for both Bedside and formal ultrasound visits

- A Subgroup is available to help sort which Clinicians do Bedside Ultrasounds:



List of which Clinicians are doing OB Prime Consult BSUS

**Check with your Supervisor or Preceptor to see if the list of Clinician's below has changed:

West Hub – All OB Providers (CNM and MD) except NPs/PAs

East Hub – All Providers (CNM and MD) except Dr. Kean and Dr. Johnson – they do **NOT** do BSUS visits
Note: Clare McBride CNM from NB has a few template days at the East side but does **NOT** do BSUS either

South Hub –

NB – MD's only

Oregon City – All Providers (CNM and MD) except Danielle Weinstein, Wendy Hall and Katie Himel – they do NOT do BSUS visits

All TN OB Providers (CNM and MD). Katie DiPietro: Has not completed all her training and is still unable to perform BSUS visits

For patients who truly have an unknown LMP

If patient is unsure of LMP, use the scripting below. The goal is to try to have the patient think through what their potential LMP is, PS team members do not need to do anything additional with the answers to the questions below.

"I am going to ask you a couple more questions regarding your last menstrual period because it is important we determine this date as this will tell us when it is appropriate to schedule you for your first few visits and ultrasound.

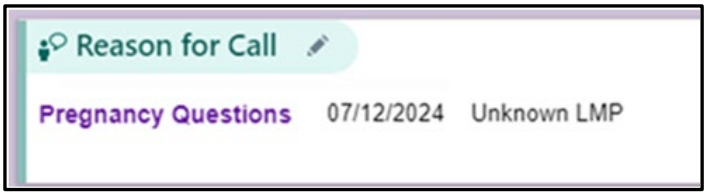
What led you to take a pregnancy test?

When do you think you conceived?"

The PS team member will then complete the NOB Intake, route that to the OB Coordinator Pool but will not schedule any NOB visits.

Last menstrual period or EDD Information: Patient is unable to provide approximate date for LMP, separate encounter sent to advice to follow up with patient within two business days. Current encounter to be routed to OB Coordinator pool for tracking, patient not scheduled for future appointments

They will then start a separate new Telephone Encounter that documents the patient has an Unknown LMP date and cannot be scheduled for their initial OB visits until further clinical instruction – use the dot phrase .*WHAUNKNOWNLMP*, use the Reason for call of "Pregnancy Questions", and then route that to the Advice Pool



Appt Notes for NOB Visits

- **OB Intake (6-8wk):** Phone visit: LMP date - Ph# xxx-xxx-xxxx

- **OB Prime Consult Bedside Ultrasound *(BSUS)* (8-10wk):** LMP date

  OR

- **OB Prim Consult Telehealth:** VIDEO Visit: Pt aware of text link, LMP Date, Ph# xxx-xxx-xxxx

- **Dating & Viability US (12 – 13.6wks):** LMP date

  *(If a patient indicates they will need to bring a child, **Staff Message** the Ultrasound Coordinator (at **WHA US Coordinators Pool**) with "CHILD at visit" in the subject line and "FYI Children attending Ultrasound visit on (date))*

- **OB Workup:** LMP date
- **Lab:** (no notes needed, leave blank)

NOB – Late to Care

Late To Care = Transfers from other providers, pregnancy resource centers or no prior OB care

Patients who are 12 - 13.6 wks GA - Walkthrough

Patients who are 14+ wks GA - Walkthrough

(12 – 13.6 weeks) Patient is late to care –

Reflective of when patient is put on the schedule, **not** the timeline of when they are calling in

Patient is not late to care (regular process) – Should be scheduled for: OB Intake Telehealth visit (6-8 weeks), OB Prime w/ BSUS visits (8-10 weeks is preferred but can scheduled up to 12 weeks), Formal OB Dating & Viability US, OB Workup, and Lab visits (12 – 13.6 weeks)

OB Transfer patients 12-13.6 weeks **should be scheduled for:** OB Intake Telehealth visit, OB Prime Telehealth (Clinician will discuss and place orders, prior to 12-week scan), Formal OB US, OB Workup, and Lab visits (12 – 13.6 weeks)

If you need help prioritizing the Formal OB US/Workup/Lab visits, contact your clinic's OB Coordinator

Patients must be seen for an **OB Prime Telehealth,** but it can be scheduled at any location with any provider if a provider at the patient's home clinic is not available *(this may vary depending on your clinic's continuity of care)*.
The OB Intake Telehealth visit can be skipped if there are no openings prior to the OB Prime Telehealth visit – if skipped, MUST Staff Message appropriate Pod Pool as an FYI

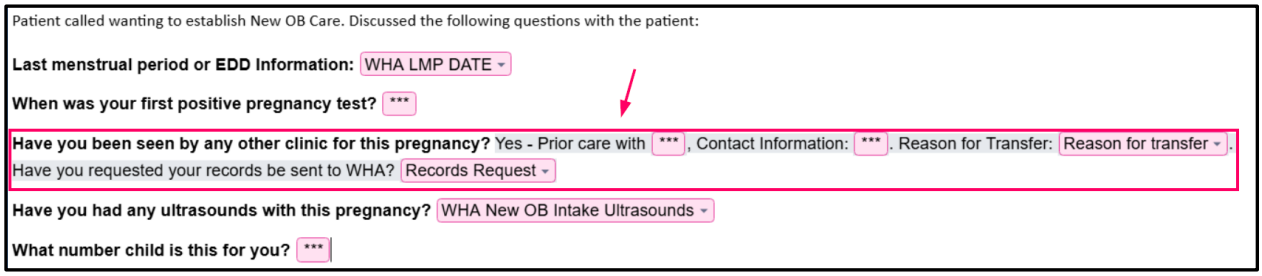
(14+ weeks) Patient is late to care / Transferring from Outside Clinic –

Reflective of when patient is calling, **not** when the patient is scheduled for their visits

If patient has an unknown LMP, follow the workflow – here

Note: It is very important that the scheduler instructs the patient to request their records be sent to our office as soon as possible and note in the appt notes that you told them. This is crucial information that our Clinicians use to better the care of the patient.

It is also asked within the NOB Intake:



Graph on how to schedule all other Late to Care 14+ patients on next page -

*Graph is assuming the patient knows at least their LMP	Est GA at time of call: 14 wks – 17.6 wks	Est GA at time of call: 18 wks – 20 wks	Est GA at time of call: 22+ wks
No prior care or only have been seen at the Pregnancy Resource center	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup within 1 week of call	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup scheduled 20wks – 22wks	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup within 1 week
Prior care but no Ultrasound for this pregnancy	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup within 1 week of call	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup scheduled 20wks – 22wks	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup within 1 week
Prior care and Ultrasound that confirmed EDD	Patient needs: • OB Intake Telehealth • OB Workup within 2 wks of call • 60 min OB US with same day OB Check scheduled 20wks – 22wks	Patient needs: • OB Intake Telehealth • 60 min OB US with same day OB Workup scheduled 20wks – 22wks	Was the US done prior to 18 wks or not an Anatomy scan? **Then patient needs:** • OB Intake Telehealth • 60 min OB US with same day OB Workup within 1 week Was the US done at or after 18 wks and was an Anatomy scan? (excluding ER USs) **Then patient needs:** • OB Intake Telehealth • OB Workup within 1 week

If the **patient declines** to be seen within recommended timeframe from the graph above, add to the Appt Notes:

Example: "*Patient declined sooner visit due to being out of town*" or "*Patient declined sooner visit due to work schedule*"

Note: **Internal Established** WHA transfer patients from clinic to clinic (excluding NWP) needs:

1. To be scheduled for a 30-minute OB Check *(except PK to HB or HB to PK only needs a 15 minute OB Check)*
2. Then Staff Message both the OB Coordinator of your clinic and the end destination clinic's OB Coordinator as an FYI

OB Triage visit type workflow

Note: Options counseling is excluded from this workflow, they will remain urgent GYN visits

Visit types: NP OB Triage, NP OB Triage Telehealth, OB Triage, & OB Triage Telehealth

Visits outside of routine OB care need to always be an OB TRIAGE visit type

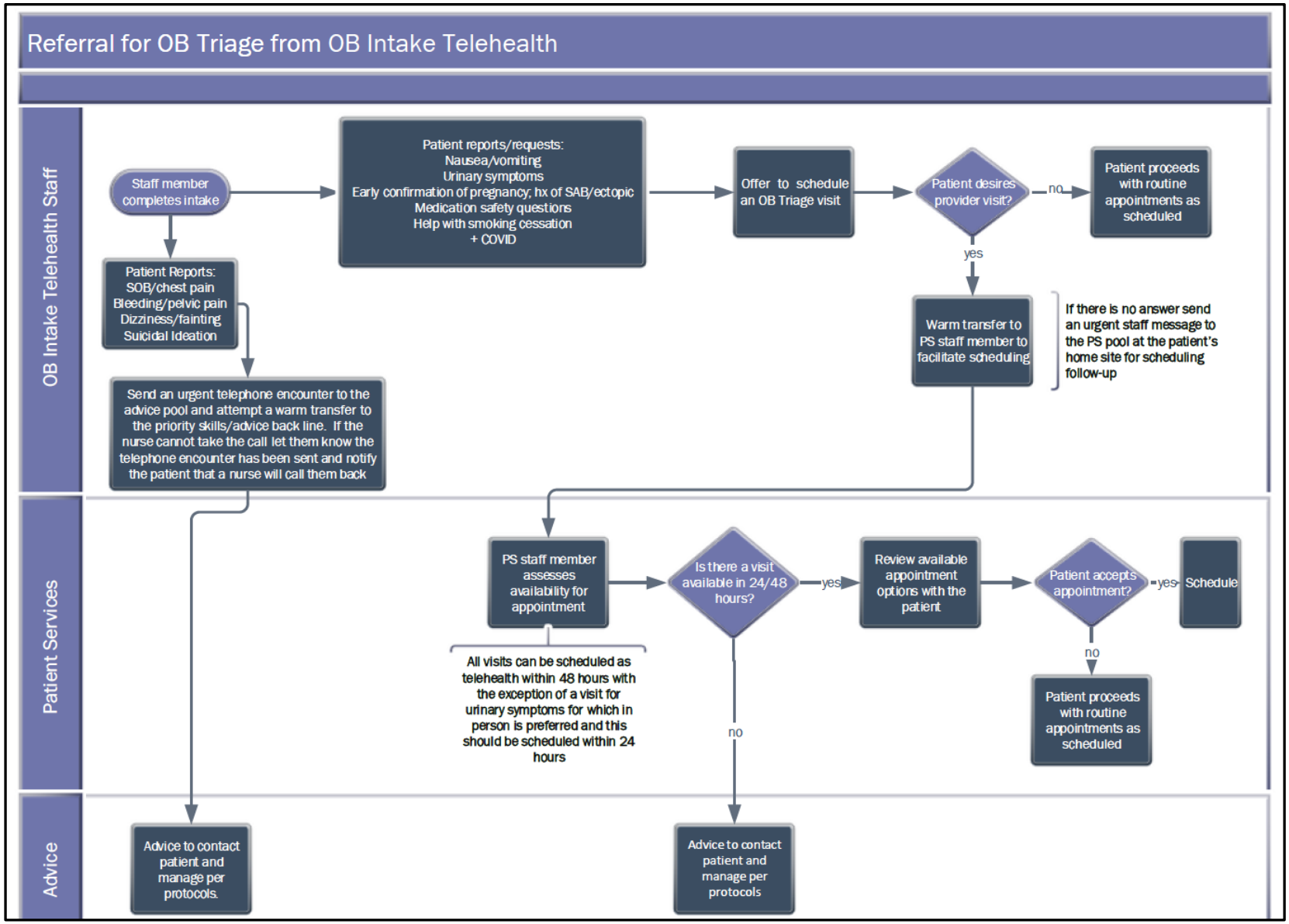
If the patient is new to WHA and has not had their OB Prime or Workup (if late to care) they are considered new and we need to use one of the NP OB Triage visit types

Examples of OB Triage use (not an all-inclusive list):

- Warm transfer from OB Intake Telehealth MA/LPN
- Directed to schedule by Advice or Clinician for non-routine OB visit/problem
- Early or Repeat Dating and Viability follow up visits

Below is the workflow map for Referrals from the OB Intake Telehealth visits (New and Est Patients) – *see next page*

Workflow Map - Referral for OB Triage from OB Intake Telehealth visit



Obtaining OB Delivery Information

Obtaining OB Deliveries

WHA OB Coordinators or a designated person at each HUB is responsible for obtaining information on which patients have delivered to get their postpartum process started. OB deliveries should be obtained every day for the day prior (Friday, Saturday, and Sunday deliveries will be obtained on Monday).

Providence Hospitals

To obtain records from Providence, you must call their specific location

Providence St Vincent: Call between 11:00am and 11:30am at 503-216-3332

Providence Portland: 503-215-5557

Providence Willamette Falls: 503-657-6723

Providence Newberg: 503-537-1758

Legacy Hospitals

Each person will create their own list within EPIC. Reference the tip sheet **Creating a patient list to obtain deliveries for Legacy facilities** on Sharepoint under the EPIC tab. Legacy hospitals: LMPH, LEMC, & LGS

Once the list in EPIC has been set up:

- Look up each patient in the Women's Health & Women's Services Unit to confirm they have a delivery summary in their Chart
- Cancel all future OB visits for patients that have delivered
- Send a Staff Message to the appropriate Pod at the patient's site to make a postpartum call
    - Subject of Staff Message: PP call needed – delivered DATE, Provider
      Example: PP call needed – Delivered 03/01/2021, Burgoine
- Ensure that the patient's OB Care Team start and end dates are updated or accurate based on delivery date and add GYN Care Team if none listed

Patient notifications about their SAB (Miscarriages) or TAB (Terminations)

Note: If the patient lets us know they had an SAB or TAB in the ER, follow this ER Visit Workflow here

Incoming notifications from Patient about SAB (Miscarriage) -

Via Phone Call:

Use Scripting, "I'm sorry to hear that. I will cancel your upcoming visits and notify your clinical team — they may reach out to you if needed. If you have any questions or symptoms you would like to discuss, please call our office and press option 5 to speak with the advice nurse team".

If the patient then requests to speak with an Advice Nurse urgently, warm transfer the call to the Priority Advice Nurse line.

If the patient requests to speak with an Advice Nurse but is not urgent, let them know that we will send a high priority note to that team and that the patient should receive a call back as soon as possible (same day).

1. After communicating the scripting above, start a Telephone Encounter marking the Reason for Conversation as **Other – Recent SAB**

2. Documentation options:
   - If during your conversation with the patient they requested to speak with an Advice Nurse not urgently, document "Patient called in and canceled future OB visits stating they had a recent SAB. Patient would like a phone call back from the Advice Nurse team" and mark the Encounter as **High Priority.**

   - If patient does **not** indicate needing a call back from the Advice Nurse team, document "No request made by patient for follow-up from Advice Nurse team at this time".

3. Route the Telephone Encounter to the appropriate Advice Nurse Pool

4. Staff Message the OB Coordinator Pool a FYI about the patient's SAB notification

Via Myhealth Appt Cancellation Message:

1. Patient Services team receives a message from the patient via Myhealth about needing to cancel their visits because of an SAB

2. PS team member will reply to patient message using .*WHASABFOLLOWUP*: "We're sorry to hear that. Your upcoming visits have been cancelled, and we will inform your clinical team as well — they may reach out to you if needed. If you have any questions or are experiencing symptoms you'd like to discuss, please call our office and press option 5 to speak with the advice nurse team."

3. Verify the "Reply directly to me" box is NOT checked

4. Cancel all visits that the patient had requested

5. Staff Message the OB Coordinator Pool a FYI about the patient's SAB notification

6. Start a Telephone Encounter from the Myhealth Cancellation request then:
   - Mark the Reason for Conversation as **Other – Recent SAB**
   - Within the documentation section add "Patient cancelled future OB visits via Myhealth stating they had a recent SAB"
   - Route the Telephone Encounter to the appropriate Advice Nurse Pool who will follow up with the patient as needed

Incoming notifications from Patient about TAB (Termination) -

Phone call or Myhealth Appt Cancellation Message:

Use Scripting: “Thank you for letting us know. If you need any follow up care or have any concerns that you would like to talk to an Advice Nurse about, please let us know”.

For Myhealth messages, use the dot phrase .*WHATABFOLLOWUP*

1. Start a Telephone Encounter marking the Reason for Conversation as **Other – Recent TAB**

2. Within the Documentation section, add “Patient cancelled future OB visits via Myhealth stating they had a recent TAB”

3. Route the Telephone Encounter to the appropriate Advice Nurse Pool who will follow up with the patient as needed

4. Staff Message the OB Coordinator Pool a FYI about the patient’s TAB notification